- Reduction in drop-out rate at school level.
- Developed attentiveness in them and thereby improved their power of comprehension.
- Improved their performance in examinations.
- Decreased the incidence of various diseases and physical disorders caused by starvation or intake of less nutritious food.
- Enabled parents to attend to their routine bread earning tasks.
- The gender difference in feeding the children reduced at home.
- A favourable attitude in parents in educating the children, specifically female children.

12.1.3. Prevention and control of anaemia

Prevention of anaemia requires approaches that address all the potential causative factors. These include:

1. **Dietary approach:** The following points need to be considered for the promotion of this strategy:
   - Promotion of consumption of pulses, green leafy vegetables, other vegetables which are rich in iron and folic acid and meat products rich in iron particularly for pregnant and lactating mothers and preschool children. Media can also be involved for creation of awareness.
   - Creation of awareness in mothers attending antenatal clinics and immunization sessions in anganwadi centres and crèches about the prevalence of anaemia, ill effects of anaemia and that it is preventable.
   - Addition of iron rich foods to the weaning foods of infants.
   - Regular consumption of foods rich in vitamin C to promote iron absorption such as orange, guava, amla, etc.
   - Promotion of home gardening to increase the availability of common iron rich foods such as green leafy vegetables.
   - Discouraging the consumption of foods and beverages like tea and tamarind that inhibit iron absorption, immediately after food especially by the vulnerable groups like pregnant women and children.
   - Promotion of iron fortified iodised salt.
2. **Supplementation:** Food based approaches through food fortification and dietary diversification are sustainable strategies for preventing iron deficiency and (Iodine Deficiency Disorder(IDD). As availability is low and dietary animal sources (haem -iron) are expensive, the key step towards addressing iron deficiency and IDD would be the implementation and scaling up of the IFA supplementation programme.

For preventing anaemia, low dosage iron is adequate. The National Anaemia Prophylaxis Programme (NAPP) in India, pregnant and lactating women receive 60 mg elemental iron+ 500 mcg folic acid (IFA tablet) daily for atleast 100 days during

UNIT-12_208-220.indd 210 23-01-2020 10:18:14